

- Sah R, Oudit GY, Nguyen TT, et al: 2002. Inhibition of calcineurin and sarcolemmal Ca^{2+} influx protects cardiac morphology and ventricular function in Kv4.2N transgenic mice. *Circulation* 105:1850-1856.
- Schwinger RH, Wang J, Frank K, et al: 1999. Reduced sodium pump $\alpha 1$, $\alpha 3$, and $\beta 1$ -isoform protein levels and Na^+ , K^+ -ATPase activity but unchanged Na^+ - Ca^{2+} exchanger protein levels in human heart failure. *Circulation* 99:2105-2112.
- Seth M, Zhang ZS, Mao L, et al: 2009. TRPC1 channels are critical for hypertrophic signaling in the heart. *Circ Res* 105:1023-1030.
- Shigekawa M, Katanosaka Y & Wakabayashi S: 2007. Regulation of the cardiac Na^+ / Ca^{2+} exchanger by calcineurin and protein kinase C. *Ann N Y Acad Sci* 1099:53-63.
- Tandan S, Wang Y, Wang TT, et al: 2009. Physical and functional interaction between calcineurin and the cardiac L-type Ca^{2+} channel. *Circ Res* 105:51-60.
- Tsuji Y, Opthof T, Yasui K, et al: 2002. Ionic mechanisms of acquired QT prolongation and torsades de pointes in rabbits with chronic complete atrioventricular block. *Circulation* 106:2012-2018.
- Valdivia CR, Chu WW, Pu J, et al: 2005. Increased late sodium current in myocytes from a canine heart failure model and from failing human heart. *J Mol Cell Cardiol* 38:475-483.
- Vindis C, D'Angelo R, Mucher E, et al: 2009. Essential role of TRPC1 channels in cardiomyoblasts hypertrophy mediated by 5-HT_{2A} serotonin receptors. *Biochem Biophys Res Commun* 391:979-983.
- Wang Z, Kutschke W, Richardson KE, et al: 2001. Electrical remodeling in pressure-overload cardiac hypertrophy: role of calcineurin. *Circulation* 104:1657-1663.
- Wang Z, Nolan B, Kutschke W, et al: 2001. Na^+ - Ca^{2+} exchanger remodeling in pressure overload cardiac hypertrophy. *J Biol Chem* 276:17706-17711.
- Wilkins BJ, Dai YS, Bueno OF, et al: 2004. Calcineurin/NFAT coupling participates in pathological, but not physiological, cardiac hypertrophy. *Circ Res* 94:110-118.
- Wu X, Eder P, Chang B, et al: 2010. TRPC channels are necessary mediators of pathologic cardiac hypertrophy. *Proc Natl Acad Sci U S A* 107:7000-7005.
- Xiao L, Coutu P, Villeneuve LR, et al: 2008. Mechanisms underlying rate-dependent remodeling of transient outward potassium current in canine ventricular myocytes. *Circ Res* 103:733-742.
- Xiong D, Heyman NS, Airey J, et al: 2009. Cardiac-specific, inducible *ClC-3* gene deletion eliminates native volume-sensitive chloride channels and produces myocardial hypertrophy in adult mice. *J Mol Cell Cardiol* 48:211-219.
- Xu X, Rials SJ, Wu Y, et al: 2001. Left ventricular hypertrophy decreases slowly but not rapidly activating delayed rectifier potassium currents of epicardial and endocardial myocytes in rabbits. *Circulation* 103:1585-1590.
- Yang KC, Foeger NC, Marionneau C, et al: 2010. Homeostatic regulation of electrical excitability in physiological cardiac hypertrophy. *J Physiol* 588:5015-5032.
- Zicha S, Maltsev VA, Nattel S, et al: 2004. Post-transcriptional alterations in the expression of cardiac Na^+ channel subunits in chronic heart failure. *J Mol Cell Cardiol* 37:91-100.

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TCM

From Atherosclerosis to Acute Coronary Syndromes: The Role of Soluble CD40 Ligand[☆]

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The binding of CD40 ligand (CD40L) to CD40 stimulates inflammatory processes including the release of proinflammatory cytokines and the expression of adhesion molecules implying a role in atherosclerosis. Patients exhibiting hypercholesterolemia, unstable angina, or acute myocardial infarction present with increased CD40L levels. Novel data suggest that elevated soluble CD40L levels not only represent a risk factor for cardiovascular disease but also predict future adverse events, especially in patients with acute coronary syndromes (ACS). Examination of the potential role of the genetic variability on CD40/CD40L genes in ACS, as regards the regulation of CD40L, appears to be of great interest. Moreover, several therapeutic approaches such as statins, antihypertensive agents, and antiplatelet agents have been suggested as potential modulators of CD40L levels anticipating a positive impact on the outcomes of patients with ACS. Whether specific agents target the CD40/CD40L system as well as its pathogenic role in ACS remains to be elucidated by large-scale studies in the future. (Trends Cardiovasc Med 2010;20:153–164) © 2010, Elsevier Inc. All rights reserved.

• Introduction

Disruption of vulnerable atheromatous plaque is the most common pathogenic

mechanism in acute coronary syndromes (ACS). There is increasing evidence that inflammation plays a key role in the pathogenesis of atherosclerosis

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